Teen Depression and Suicide Prevention

108

SUICIDAL BEHAVIOR IN ADOLESCENTS

Introduction

- ☑ Suicide is the third leading cause of adolescent mortality in India and the fifth leading cause, globally.
- ☑ Every hour one child dies of suicide in India.
- ☑ Most adolescent suicides are impulsive. They commit suicide mainly by hanging, poisoning, drowning, and self-immolation.
- ☑ The important causes are family problems, break-up in romantic relationships, physical and mental illness, failure in examinations, poverty, bereavement, hero worship, unemployment, gender discrimination, and mental disorders.
- ☑ Nonsuicidal self-injury (NSSI) behavior is deliberate destruction or alteration of body tissue without suicidal intent. Adolescents indulge in NSSI to cope with mental distress, to secure attention and as a form of self-punishment. NSSI is a risk factor for suicide.
- ☑ Due to stigma attached to seeking mental healthcare and shortage of child and adolescent psychiatrists, pediatricians are often the first point of contact in the healthcare system for adolescents.

Documentation and medical record keeping is essential.

- ☑ Emergency medical services, wound care, and appropriate immunization to be ensured for those presenting with life-threatening conditions and with injuries.
- ☑ *Suicide screening:* During a healthcare visit, all adolescents should be evaluated for suicide risk during home environment, education and employment, eating, peer-related activities, drugs (HEEADSSS) psychosocial screening. Disclosure of suicidality entails sharing this information with a trustworthy caregiver that the adolescent chooses. The probe questions are:

  Sometimes when things get difficult, young people think of death and hurting/killing themselves. Have you had any such thoughts?

  If yes, ask, have you actually tried hurting/killing yourself anytime?

  If no, enquire about coping and stress management skills and praise the adolescent for his/her resilience.
- ☑ *Risk assessment:* If suicide screen is positive:
  - Assess severity and immediacy of risk. Short nonjudgmental questions are asked to assess intent, thoughts, and plan for suicide. "IS PATH WARM" mnemonic can be used to identify the warning signs of suicide **(Table 1)**.
  - Assess risk and protective factors **(Table 2)**.

TABLE 1: Warning signs of suicide: IS PATH WARM.

I	Ideation (talking or writing or posting online about death, threatening to hurt or kill self, and giving away prized possessions)
S	Substance use
P	Purposelessness (having no reason to live)
A	Anxiety
T	Trapped
H	Hopelessness
W	Withdrawal
A	Anger (uncontrolled)
R	Recklessness
M	Mood changes

TABLE 2: Risk and protective factors for adolescent suicide.

Risk factors	***Protective factors***
Individual	
☑ Female sex ☑ School drop out ☑ Previous history of suicidal attempt ☑ Death wish/suicidal notes/online posts ☑ Child abuse/trafficking, bullying, cyberbullying ☑ Nonsuicidal self-injury ☑ Mental and physical illness ☑ Failed intimate relationship ☑ Substance use disorder ☑ Children in conflict with law and in need of care and protection ☑ Gender minority youth: LGBQTIA + marginalized youth	☑ Positive coping skills ☑ Emotional self-regulation skill ☑ High self-esteem ☑ Conflict resolution skill ☑ Involvement in hobbies and activities ☑ Employment ☑ Religious belief ☑ Good social skills ☑ Help seeking behavior
School and peers	
☑ Failure in examination ☑ Learning problems ☑ Violent peers ☑ Lack of school counseling services and social support	☑ Academic achievement ☑ Positive peer relationships ☑ Strong school connectedness ☑ Life skill education, suicide and bullying prevention programs
Family	
☑ Family dysfunction and violence ☑ Child abuse and neglect ☑ Economic crisis, environmental disasters ☑ Family history of suicide, mental disorder, alcohol use disorder	☑ Family stability ☑ Authoritative parenting ☑ Strong family connectedness ☑ Positive discipline
Community	
☑ Access to means of suicide ☑ Unsafe media portrayal of suicide	☑ Access to adolescent friendly and mental health services ☑ Responsible media reporting ☑ Comprehensive national policy for suicide prevention

Management of Adolescent Suicidal Behavior

Adolescents with suicidal thoughts care classified into three groups:

1. Low risk—suicidal ideation, thoughts of death only, and no plan or behavior.
2. Moderate risk—suicidal thoughts, limited suicidal intent, and no clear plan.
3. High risk—suicide plan with preparatory behavior, past history of suicide attempt, NSSI, and multiple risk factors.

☑ *Urgency of referral*:

- Emergency mental health referral is needed if there is immediate threat to life, for patients assessed as moderate-to-high risk and with severe mental distress.
- Urgent mental health referral within 48–72 hours is needed for patients with low risk.

☑ *First responder emergency clinical care:* After providing the requisite emergency medical care and arranging for a referral to a psychiatrist, the pediatrician should:

- Impart psychoeducation and counseling that enables the adolescent and the family to foster hope and counter the feelings of hopelessness, helplessness, worthlessness, shame, and guilt.
- Activate psychosocial support and discuss lethal means counseling to ensure safety and restrict access to means of suicide (e.g., medication and knife).
- Create a safety plan with adolescent and the family which outlines action plan of a future situation of suicidal intent, emergency contact numbers of family members, health professionals, suicide help line (e.g., 1098), and builds self-worth.

☑ *Follow-up:* Pediatricians should collaborate with psychiatrists, psychologists, social workers, educators, and counselors to formulate a follow-up plan. This includes life skills training, building resilience, and therapy for mental disorders along with training on positive parenting. Follow-ups are scheduled once a week for 2 months; later once a month in the first 1 year and twice in the second year.

Prevention of Adolescent Suicidal Behavior

☑ Prevention strategies in clinical, school, and community settings; use life skill education approaches to fostering resilience; and to promote peer, family, and school connectedness.

☑ These also aim at helping those detected at risk for suicide, screening, and treating mental disorders and preventing recurrences.

☑ Pediatricians should advocate for a national comprehensive suicide prevention policy with special focus on the adolescent age group.

ADOLESCENT DEPRESSION

Introduction

Depression is a common mental disorder. It should be differentiated from normative mood swings of adolescence. The latter are short lasting, not more than a few hours in a day and do not jeopardize the normal functioning of the teen.

Magnitude of Adolescent Depression in India

- ☑ It has an incidence of 4–8% in general population. About 15% of adolescents who seek medical help get diagnosed as depression while around 36% of them report persistent sadness.
- ☑ Adolescent depression carries risks of social exclusion, discrimination, poor academic performance, risk-taking behaviors, physical ill-health, self-harm/suicidality, and human rights violations leading to an underproductive adult life.

Diagnostic Criteria

According to the Diagnostic Statistical Manual of Mental Disorders 5th edition (DSM-5) criteria for diagnosis of adolescent depression:

- ☑ At least one of the two core criteria: Low or irritable mood and loss of pleasure
- ☑ At least four out of the following:
  - Significant weight gain or loss (when not dieting) (change of >5% of body weight in a month)
  - Decrease or increase in appetite
  - Insomnia or hypersomnia
  - Psychomotor agitation or retardation
  - Feelings of worthlessness or excessive or inappropriate guilt
  - Diminished ability to think or concentrate, or indecisiveness
  - Recurrent thoughts of death (not just fear of dying), recurrent suicidal ideation/plan/or an attempt
- ☑ These symptoms should be *present for at least 2 weeks, on most of the days*. They could be subjective or observed by the caretakers/friends and should cause a significant functional impairment in spheres of daily living like social, academic or occupational.

Evaluation

Adolescents with depression present with one or more of the physical, emotional, behavioral, or cognitive problems listed earlier and/or with social withdrawal, irritability, and poor school performance.

The home environment, education/employment, eating, activities, drugs, sexuality, suicide/depression, and safety (HEEADSSS) history along with past and family history of psychiatric issues should be elicited. A history of manic episode with over talkativeness, overconfidence, and delusions of grandiose should be ruled out. History of irrelevant talk, hallucinations, and delusions indicate psychosis. Adolescents with depression should be screened for comorbid mental disorders **(Table 1)**.

It needs to be differentiated from mood changes secondary to organic problems such as anemia, hypothyroidism, Wilson's disease by doing a detailed clinical assessment, hemogram, thyroid profile, liver function tests, and other relevant investigations.

Patient Name_____ Date of visit _____

Over the past 2 weeks, how often have you been bothered by any of the following problems?	Not at all	Several days	More than half the days	Nearly every day
1. Little interest or pleasure in doing things	0	1	2	3
2. Feeling down, depressed or hopless	0	1	2	3

Fig. 1: Patient Health Questionnaire-2 (PHQ-2).

Psychometric tests that can be used in office practice are Patient Health Questionnaire-2 (PHQ-2) **(Fig. 1)** and Beck's Depression Inventory-II (BDI-II). A cutoff of 3 on PHQ and 14 on BDI-II, mandate detailed evaluation with the DSM-5 criteria. A BDI-II score of 14–19 indicates mild, 20–28 moderate, and 29–63 indicates severe depression. Patient Health Questionnaire-9 modified for adolescents (PHQ-A) can also be used for screening. PHQ-A and BDI-II can be used to monitor the progression of the disease.

TABLE 1: Comorbidities and their clinical pointers.

Comorbidities	*Clinical pointers*
Anxiety disorders	Restlessness, perspiration, palpitations, avoidant behavior, disproportionate worrying, and repetitive thoughts/actions
Eating disorders	Thin build, excessive exercise, extreme calorie consciousness, purging, acid stains on teeth, and menstrual irregularities
Substance use disorder	Smell/stains on teeth and clothing, suspicious behavior, demanding money, and injection marks
ADHD	Early onset PSP, easy distractibility, carelessness, racing of thoughts, squirming, impatience, and inattention
Learning disability	Difficulty in reading/writing/math, spelling mistakes, scores better in oral exams than written ones, and IQ is normal
Conduct disorders	Defiance to authority figures, deliberate harm to animals, persons and properties, and noncompliance to rules and laws
Bullying/abuse	Physical injuries, low self-esteem, withdrawn, apprehensive, avoidant behavior, aggression, and sexual misconduct
Problematic Internet use	Excessive gadget use, irritated when gadget taken away, daily routine, and schooling and social interaction affected

(ADHD: attention deficit hyperactivity disorder; PSP: poor school performance)

Treatment

The modalities of treatment are pharmacological and/or psychotherapy. Mild depression generally responds to counseling and psychotherapy.

A supportive framework of positive lifestyle (scheduling of the day, healthy diet, healthy media use, physical exercises/sports, hobbies, and relaxation techniques, such as deep breathing, yoga, and mindfulness), philanthropic activities, counting the blessings, positive self-talks, and the presence of supportive caregivers, teachers, and peers, facilitates recovery.

Counseling involves empathetic listening while allowing the adolescents to ventilate their feelings, gently leading them to appreciate their strengths, and motivating them to use these strengths to resolve and cope with problems. Adolescents are encouraged to maintain a mood diary and journal. Cognitive behavior therapy (CBT) or rational emotive behavioral therapy (REBT) are based on the premise that negative thoughts are the root cause of the sad mood and such thoughts can be converted systematically into useful positive thoughts through multiple sessions with the trained psychotherapists.

Pharmacotherapy

For management of moderate and severe depression, the drugs of choice are selective serotonin reuptake inhibitors (SSRIs) **(Table 2)**. They are scaled up gradually starting from a small dose and need to be titrated depending on the response (which usually begins after 2 weeks). SSRIs are to be continued for a period of 6–12 months and then slowly tapered off over 4–6 weeks. The adolescents on SSRIs have to be closely monitored for suicidal behavior. Combination of SSRI and CBT gives the best outcome in cases of moderate depression.

Pediatricians should refer cases of severe depression or those with suicidality and/or signs of psychoses or other major comorbidities to the psychiatrist.

TABLE 2: Pharmacotherapy in adolescent depression.

Pharmaceutical agent	*Type*	*Dose*	*Side effects*
Etizolam	Short-acting anxiolytic	0.25 mg M, 0.5 mg HS	Muscle relaxation
Clonazepam	Short-acting anxiolytic	0.25 mg M, 0.5 mg HS	Drowsiness, habit forming
Fluoxetine (SSRI)	Antidepressant/anxiolytic	10–20 mg OD	Sleep and GI issues aggression, mania
Escitalopram (SSRI)	Antidepressant	10 mg OD	Sleep and GI issues aggression, mania

(GI: gastrointestinal; SSRI: selective serotonin reuptake inhibitor)

Suicidal Behavior in Adolescents

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Adolescent Depression

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